

MEMORIAL RIVERVIEW HOSPITAL

Department of Internal Medicine | 123 Elm Street, Springfield, IL 62704

Patient:	Marcus J. Whitfield	DOB:	14/03/1968 (Age 58)
MRN:	0047-2291-XR	Date of Letter:	28/08/2026
Referring Physician:	Dr. Angela Torres, MD	Consulting Physician:	Dr. Rajiv Menon, MD (Cardiology)

Dear Dr. Torres,

Thank you for referring Mr. Whitfield for cardiology evaluation following his admission on 22/08/2026. He presented in hypertensive urgency, with a blood pressure of 198/124 mmHg on arrival, which has since resolved with treatment. During this admission he was also newly diagnosed with Type 2 Diabetes Mellitus, based on an HbA1c of 8.9% and a fasting glucose of 216 mg/dL, and further workup revealed Stage 2 Chronic Kidney Disease with an eGFR of 68 mL/min/1.73m², along with mild left ventricular hypertrophy noted on echocardiogram.

His past medical history is notable for essential hypertension diagnosed in 2015 and hyperlipidemia diagnosed in 2017. He also has a history of gout, with his last flare in 2021, and underwent an appendectomy in 1989. Of note, his father died of a myocardial infarction at age 61, which is relevant given his current presentation.

At the time of discharge on 27/08/2026, Mr. Whitfield was hemodynamically stable, with a blood pressure of 138/86 mmHg, heart rate of 78 bpm and regular, respiratory rate of 16 per minute, temperature of 36.7°C, and oxygen saturation of 98% on room air. He weighs 94.2 kg and is 178 cm tall, giving a BMI of 29.7.

During his stay, he underwent a 12-lead ECG on 22/08/2026, which showed sinus rhythm without acute ST changes. A transthoracic echocardiogram performed the following day, on 23/08/2026, demonstrated an LVEF of 55% with mild LVH and no valvular abnormality. A renal ultrasound on 24/08/2026 showed no hydronephrosis, though there was mild cortical thinning noted bilaterally.

He has been started on amlodipine 10 mg orally once daily and losartan 100 mg orally once daily for blood pressure control, along with metformin 500 mg orally twice daily, which is being titrated up to 1000 mg twice daily over the next two weeks for his newly diagnosed diabetes. He remains on atorvastatin 40 mg orally at night for hyperlipidemia and aspirin 81 mg orally daily.

We discussed the potential side effects of these medications with the patient in detail. With amlodipine, he may experience peripheral edema, flushing, or dizziness. Metformin can cause gastrointestinal upset and diarrhea, and carries a rare risk of lactic acidosis, so he was advised to hold the medication prior to any contrast imaging studies. Atorvastatin may cause myalgia and, rarely, hepatotoxicity, so we will be checking his liver function tests in six weeks. Losartan carries risks of hyperkalemia and hypotension, and although cough is less common than with ACE inhibitors, this was also mentioned.

Mr. Whitfield was counseled extensively on warning symptoms that should prompt him to seek immediate medical attention. These include chest pain, pressure, or tightness lasting more than five minutes, sudden

shortness of breath or orthopnea, and blurred vision, severe headache, or confusion, which could indicate a hypertensive emergency. He was also warned about symptoms of hypoglycemia, such as a blood glucose below 70 mg/dL accompanied by sweating, tremor, or confusion, as well as muscle weakness or dark urine, which could signal rhabdomyolysis related to his statin therapy. Finally, he was told to seek urgent care for any swelling of the face or tongue or difficulty breathing, which could represent angioedema.

Going forward, we plan to see him back in cardiology clinic in four weeks, with repeat HbA1c and renal panel testing in three months. He has been asked to monitor his blood pressure at home twice daily and bring his log to the next visit. A dietary consult has been arranged to support him with diabetic and low-sodium dietary changes, and we will continue his low-dose aspirin while reassessing his bleeding risk on an annual basis.

Please do not hesitate to contact our office with any questions.

Sincerely,

Dr. Rajiv Menon, MD

Cardiology, Memorial Riverview Hospital